

HEALTHCARE- HOSPITALS INDUSTRY

Overview

- India spends little on health but is formalising fast.** The investable opportunity is not the headline market — it is a ₹78,500 cr organised private hospital pool that is consolidating, premiumising into complex specialties, and being re-rated on revenue per bed rather than number of beds.
- Valuation:** India's healthcare market was valued at **US\$ 400 bn in 2024** and is projected to reach **US\$ 638bn by 2025**
 - Projection:** The healthcare market is expected to expand at a compound annual growth rate (CAGR) of **8% from 2024 to 2032**.
 - Production:** India is the world's largest provider of generic medicines by volume, holding a **20% share of global pharmaceutical exports**.
 - Volume & Value:** The India medical equipment market size reached **US\$ 28.63bn in 2024** and is projected to reach **US\$ 60.38bn by 2033**
 - Contribution:** The pharmaceutical sector currently contributes around **1.72% of the country's GDP**

Industry Size & Growth

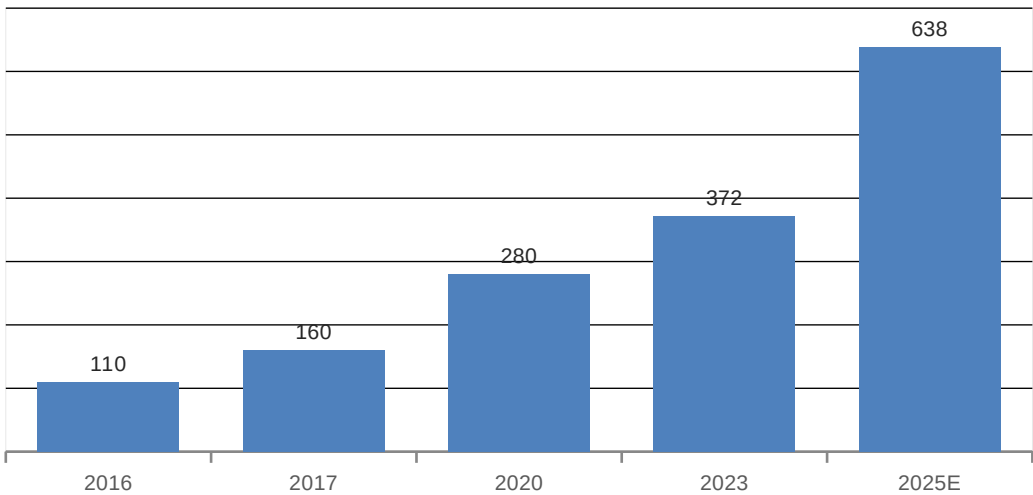
US\$ 372 bn
FY2025E

- India healthcare (broad) valued at US\$ 372 bn in 2023; hospitals alone are ~US\$ 135 bn in FY26, growing ~10.6% to US\$ 202 bn by 2030.
- Organised private hospitals: revenue pool ₹78,500 cr (FY25, 98 hospitals ≈ two-thirds of sector). CRISIL forecasts 14-15% growth in FY27 — a fifth straight double-digit year.
- Health spend is only ~3.3% of GDP vs ~9.9% OECD average — the structural runway argument.

Industry Size & Growth

Sub-market	Current size	Forecast	CAGR
Hospitals	US\$ 135.3 bn (FY26)	US\$ 202.5 bn by 2030	~10.6%
Diagnostics	₹ 441 bn (FY23)	₹ 700-755 bn by FY28E	~11-12%
Medical devices	US\$ 18.3 bn (2026)	US\$ 26.7 bn by 2031	~7.8%
Health insurance	₹ 614 bn premium (FY25)	18% CAGR FY18-25	~18%
Medical value travel	US\$ 8.7 bn (2025)	US\$ 16.2 bn by 2030	~13.2%

Indian healthcare market (US\$ bn, broad definition)



CAGR 2016-25E: ~22% | "Broad definition" includes hospitals, pharmaceuticals, diagnostics, devices, insurance and telemedicine — always caveat this in an interview.

Emerging Trends

% of Public Expenditure: India's public expenditure on healthcare is expected to be 1.9 % of GDP in FY26, compared to 2.5% in FY25, as per the Economic Survey 2024-25

Credit Incentive Programme: The Indian government is planning to introduce a credit incentive programme worth Rs. 50,000 crore (US\$ 6.8 billion) to boost the country's healthcare infrastructure.

Significant Demand: The healthcare sector, as of FY24, is one of India's largest employers, employing a total of 7.5 million people.

Scheme Announcement: The government announced Rs. 9,406 crore (US\$ 1.08 billion) outlay for PMJAY in the Union Budget FY26, an increase of 28.8% from budget FY25.

Telemedicine: The telemedicine market is expected to reach US\$ 5.4bn by FY25, driven by increased demand for remote healthcare solutions & advancements in technology.

~65%

Sector occupancy forecast, FY27

20-21%

Operating margin sustained, FY27

₹52,200

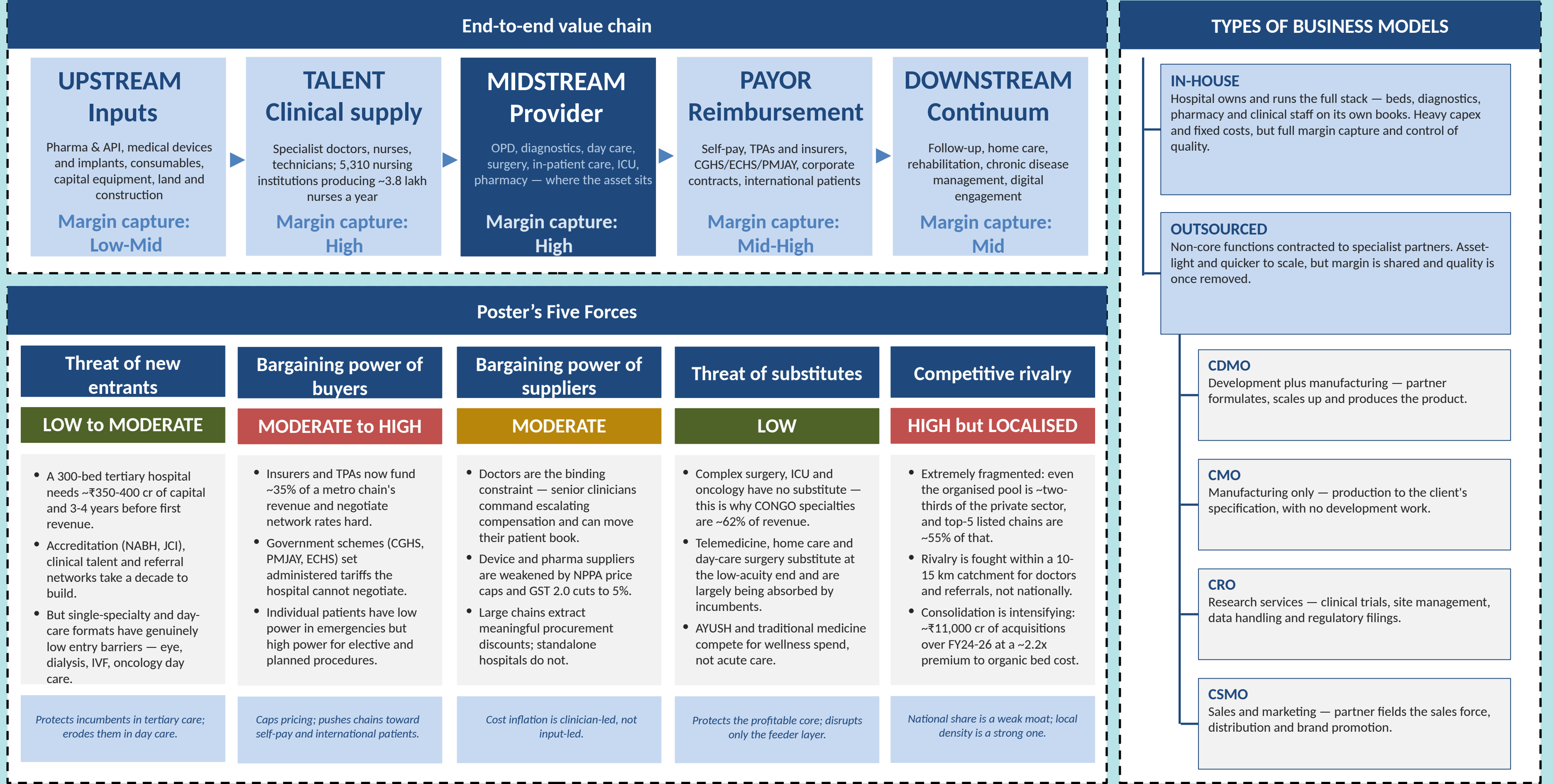
ARPOB per day forecast, FY27

10,000

Organic beds live over FY26-27

References: IBEF Healthcare (Feb-26, citing Frost & Sullivan); CRISIL Ratings (Feb-26); CRISIL MI&A; IRDAI; Mordor Intelligence (Jun-26); MoHFW NHA 2022-23; OECD Health Statistics

HEALTHCARE- HOSPITALS INDUSTRY



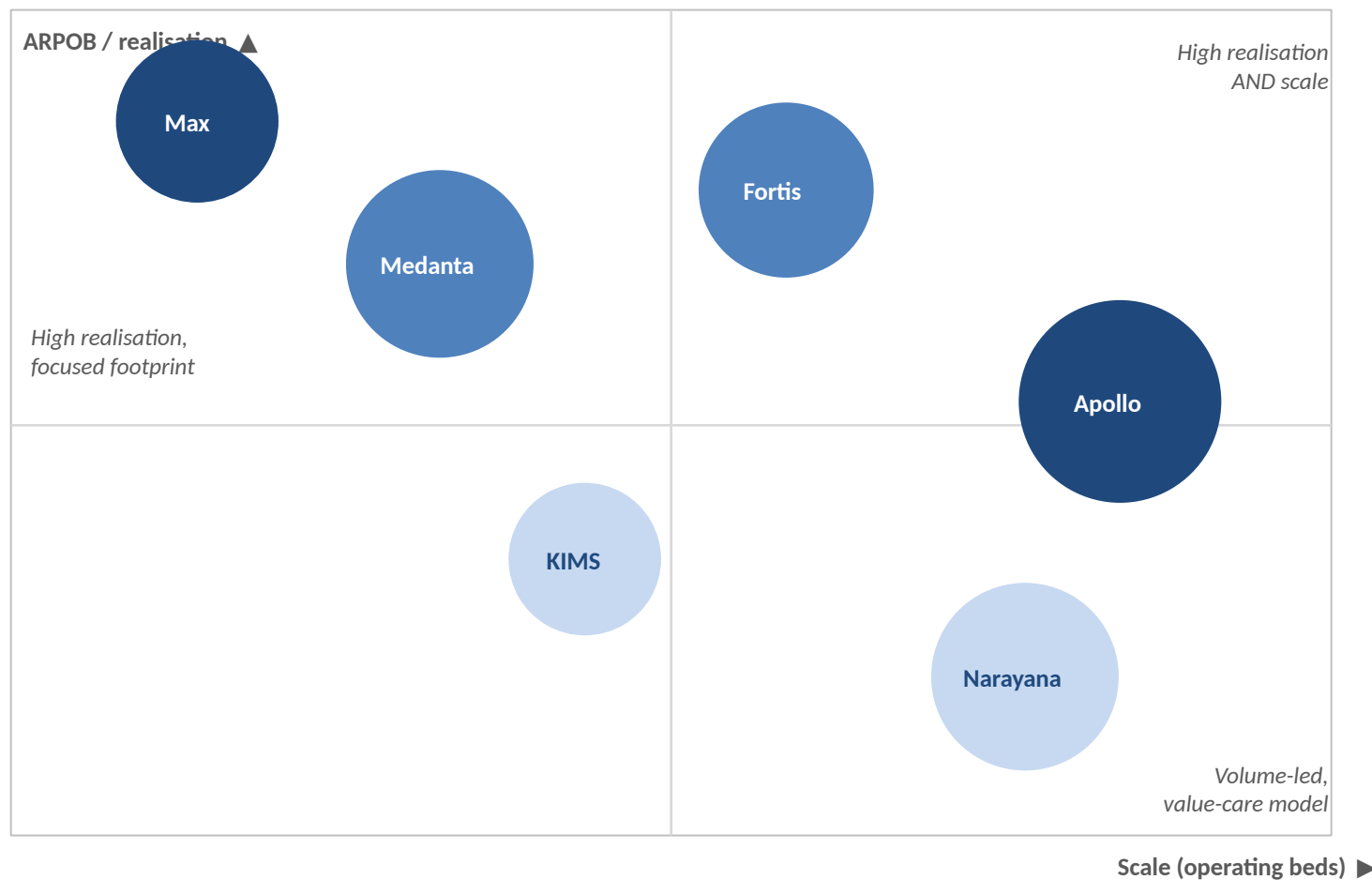
HEALTHCARE- HOSPITALS INDUSTRY

Competitive landscape

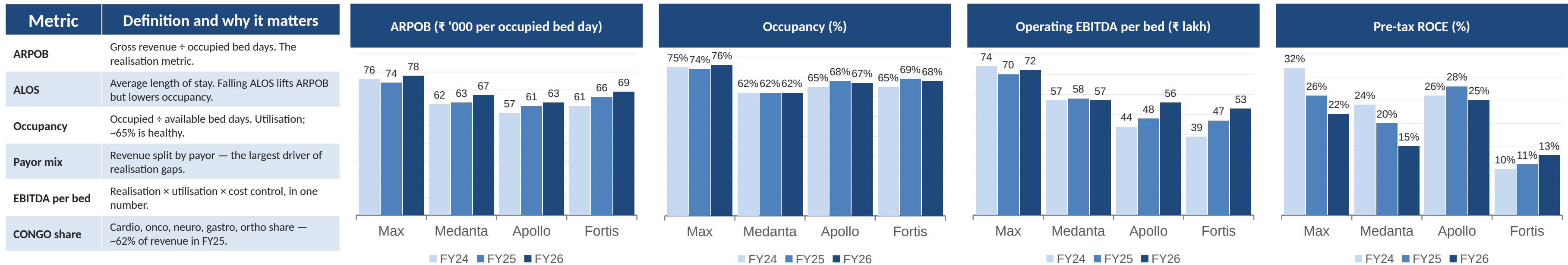
Company	Beds	Revenue (₹ cr)	EBITDA (₹ cr)	EBITDA margin	PAT (₹ cr)	Positioning
Apollo Hospitals	8,131	25,229	3,769	14.9%	1,942	Integrated: hospitals + pharmacy + digital
Max Healthcare	6,000+	10,538	2,638	26.2%	1,631	Metro-dense, quaternary, highest realisation
Fortis Healthcare	~5,800	9,128	2,085	22.8%	1,064	Hospitals + Agilus diagnostics; margin recovery story
Narayana Health	~5,600	7,896	1,793	22.7%	937	Value care in India; Cayman & UK internationally
Global Health (Medanta)	~3,700	4,508	1,056	24.2%	554	Pure-play quaternary; net cash balance sheet

References: Company FY26 results releases and investor presentations (May-26); Max Healthcare investor presentation (26-May-26) for peer operating metrics; CRISIL Ratings (Feb-26)

Positioning



Operating Benchmarks



HEALTHCARE- HOSPITALS INDUSTRY

Capacity Intensity	Industry Size & Growth				KEY DRIVERS		
Greenfield Brownfield Acquisition Asset-light formats — operations & management contracts, built-to-suit long leases and long-term service agreements — let a chain add beds with almost no balance-sheet capital. Max has executed these for ~1,230 beds across Mohali, Thane, Dehradun and Pitampura.	Beds Installed capacity	× Occupancy ~65% sector	× ARPOB ₹45k-78k / day	= Revenue Then – cost base	Revenue Drivers Rising Disease Burden Health Insurance Penetration Medical Tourism Technology Advancement Outpatient & Inpatient	Cost Drivers Human Resources Medical Equipment Drugs & Consumables Drugs & Consumables Underutilization of Assets	Growth Drivers Demographic Shifts Rising Disposable Income Govt Initiatives Expansion into Tier 2 & 3 cities Digital Transformation
	- In-patient (IP) is 70-80% of hospital revenue and nearly all of the profit; out-patient (OP) is a low-margin feeder that generates the surgical funnel. - Drugs, implants and consumables are billed through at modest mark-up — high revenue, low margin, and increasingly price-capped by NPPA. - Adjacencies are deliberately capital-light: non-captive pathology (Max Lab), home care (Max@Home), pharmacy and digital consults extend the brand without new beds.						

Cost structure and operating leverage				Working capital: the quiet differentiator			
				  			
Cost line	% of net revenue	Behaviour	Lever	Company	Debtor days	Working capital	Read-through
Direct costs — drugs, consumables, implants, clinician fees	41.0%	Largely variable	Procurement scale, case mix	Apollo Hospitals	51	Negative (–6 days)	Pharmacy float funds the hospital
Indirect overheads — salaries, power, maintenance, admin	32.8%	Largely fixed	Occupancy, ALOS reduction	Max Healthcare	87	Positive	Institutional & international mix lengthens DSO
Operating EBITDA	26.2%	Leverage-driven	Every incremental patient is high-margin	Dr Lal PathLabs	Low, B2C	Negative (–26 days)	Diagnostics is a cash business — structurally superior
Depreciation & amortisation	5.0%	Fixed	Asset-light formats reduce this				
Profit after tax	16.2%	—	Tax rate ~21% normalised				
Illustrated using Max Healthcare network FY26 — the most granular public cost disclosure in the sector. Direct cost rose 160 bps YoY on higher doctor compensation, the sector's single biggest margin risk.				Apollo's cash conversion cycle is negative because pharmacy suppliers are paid after patients pay — a genuine competitive advantage that is easy to miss if you only look at the P&L. References: Company FY26 investor presentations (Max, Apollo, Fortis, Narayana, Global Health); CRISIL Ratings (Feb-26); Screener.in (balance sheet ratios)			

HEALTHCARE- HOSPITALS INDUSTRY

Balance sheet | position as at 31 March 2026 unless stated

₹ crore	Apollo (FY25)	Max (FY26)	Fortis (FY26)	Narayana (FY26)	Medanta (FY26)	Interpretation
Shareholders' equity	8,212	12,088	~10,150	~4,570	~4,600	Max is now the most capitalised pure hospital balance sheet
Gross debt	7,864	2,924	~3,000	~3,100	631	Apollo debt includes pharmacy and digital businesses
Cash & equivalents	2,487	1,122	~670	~860	1,222	Medanta is the only net-cash operator
Net debt	~5,380	1,908	2,334	2,240	(591)	Narayana's rose sharply on the UK acquisition
Net debt / equity	~0.65x	0.16x	0.23x	0.49x	Net cash	All comfortably inside covenant territory
Net debt / EBITDA	~0.9x	~0.72x	1.09x	~1.25x	Net cash	Sector average ~1.7x — the listed set is well inside it
Pre-tax ROCE	16.6%	22%	13%	n.d.	14.9%	Medanta ex-Noida and projects is 23.4%
Return on equity	18.4%	~13.5%	~10.5%	~17.7%	~12.0%	Depressed where fresh equity has not yet been deployed
Debtor days	51	87	n.d.	n.d.	n.d.	Driven by institutional and international payor share
Working capital	~6 days	Positive	Positive	Positive	Positive	Apollo's pharmacy float is a structural advantage

Figures marked ~ are derived from disclosed ratios (for example Fortis equity is implied from net debt of ₹2,334 cr at a net debt to equity ratio of 0.23x). Apollo's balance sheet is the last full published consolidated position (FY25); its FY26 net debt / EBITDA is ~0.9x. "n.d." = not disclosed.

The economics of a single occupied bed, per year

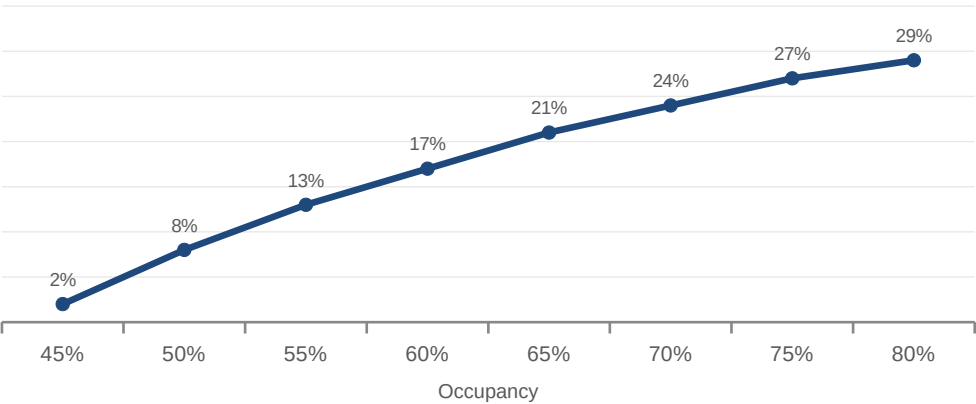
Unit Economics	
Gross revenue per occupied bed day (sector, FY27E)	₹ 52,200
Occupied bed days per year at ~65% occupancy	× 237 days
Annual revenue per installed bed	₹ 1.24 cr
Less: direct costs — drugs, implants, clinician fees (41%)	(₹ 0.51 cr)
Contribution	₹ 0.73 cr
Less: fixed overheads — salaries, power, admin (33%)	(₹ 0.41 cr)
Operating EBITDA per installed bed	₹ 0.32 cr

Illustrative build using sector-level benchmarks, not a single company. Max's disclosed operating EBITDA per bed of ₹72 lakh sits well above this because it runs 76% occupancy at ₹78,000 ARPOB; Fortis at ₹53 lakh sits close to it.

Payback and operating leverage

Route	Capital per bed	EBITDA per bed	Simple payback	Time to breakeven
Organic (greenfield / brownfield)	~₹1.2 cr	~₹0.32 cr	~3.8 years	12-18 months
Acquisition of an operating asset	~₹2.6 cr	~₹0.32 cr	~8.1 years	Immediate
Asset-light (O&M / built-to-suit lease)	Minimal	Lower, rent-adjusted	Not capital-bound	~6 months (Max Dwarka)

Operating leverage: why occupancy is everything



Illustrative, holding ARPOB and the fixed cost block constant. Because roughly a third of the cost base does not vary with volume, each 5 percentage points of occupancy adds 3-5 points of EBITDA margin near the operating point — and below roughly 45% occupancy a tertiary hospital does not cover its fixed costs at all.

- The 2.2x acquisition premium is only justified if the acquirer can lift the target's ARPOB and occupancy toward its own — which is precisely the Max and Manipal thesis, and precisely the execution risk.
- The shift in breakeven from 3-4 years to 12-18 months is the most important structural change in the sector's economics this decade. It halves the ROCE drag from expansion.

References: Max Healthcare network FY26 cost structure; CRISIL Ratings (Feb-26) for ARPOB, breakeven and bed-cost benchmarks; company FY26 disclosures

References: Company FY26 results releases and investor presentations; Max Healthcare network balance sheet (Mar-26); Screener.in (Apollo FY25 ratios); CRISIL Ratings (Feb-26)

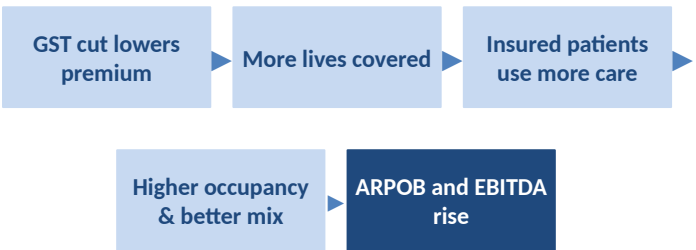
HEALTHCARE- HOSPITALS INDUSTRY

Key business risks

Category	Risk	Probability	Severity	Mitigation in practice
Regulatory	Price caps extended from stents and implants to whole procedures, or state tariff controls under the Clinical Establishments Act	Medium	High	Shift case mix to quaternary and self-pay; diversify into international patients
Operational	Clinician pay inflation — direct costs rose 160 bps of revenue at Max in FY26, largely on doctor compensation	High	Medium	Equity-linked retention, in-house DNB pipelines, salaried rather than fee-share models
Execution	New capacity fails to ramp — ~10,000 beds arriving over FY26-27 into partly untested catchments	Medium	High	Brownfield bias, asset-light leases, phased commissioning; Medanta Noida is the live test case
Competitive	Overpaying for acquisitions at ~2.2x organic bed cost as targets become scarcer	Medium	Medium	Explicit return thresholds — Max targets 20-25% ROCE within four years of an acquisition
Financial	Receivable stretch from TPAs and government schemes; FX debt at internationally expanded players	Medium	Medium	Payor diversification; Narayana carries US\$117 mn and £150 mn of FX exposure
Geopolitical	Medical value travel disrupted by instability in source markets — the Bangladesh episode cut volumes	Medium	Medium	Broaden source geography into Africa, Central Asia and the Gulf; Max runs 15 centres in 14 countries
Technological	Equipment obsolescence and cyber-security exposure across digitised clinical systems	Medium	Low	Phased capex, ISO 27001 certification, cyber insurance, network segmentation
Macroeconomic	Slower income growth or insurance penetration stalling after the GST-driven uplift fades	Low	Medium	Healthcare demand is largely non-discretionary; elective procedures are the cyclical part
ESG / reputational	Public and political backlash over hospital billing — the usual precursor to price regulation	Medium	High	Transparent packages, charitable treatment quotas, accreditation and published outcomes

Insurance and the payment pools

The mechanism: how an insurance rupee becomes a hospital rupee



This is the causal chain to draw on a whiteboard if asked why the GST change matters to hospital stocks rather than only to insurers.

The three payment pools and what each does for a hospital

PRIVATE INSURANCE & CORPORATE

- GST cut from 18% to nil on individual and family policies from 22 September 2025 — the single largest affordability improvement in the market's history.
- Standalone health insurers hold ~41% of the non-life market, up from ~30% in FY20.
- For hospitals: negotiated network rates, 45-90 day receivables, and the fastest-growing volume pool.

GOVERNMENT SCHEMES

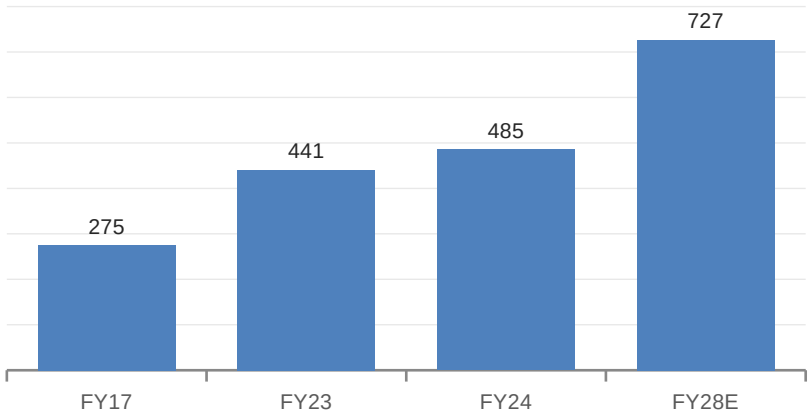
- Ayushman Bharat PM-JAY: 42.7 crore cards issued; 32,544 hospitals empanelled including 15,507 private.
- CGHS tariff revision materially improves procedure viability and accelerates new-unit breakeven.
- For hospitals: fills beds at the lowest realisation, with 90-180 day receivables. Volume, not margin.

OUT-OF-POCKET

- Still 43.4% of total health expenditure, down from 64.2% a decade ago — the decline is the growth story.
- Highest realisation and immediate payment; concentrated in metros and in elective and preventive care.
- For hospitals: the most profitable rupee, and the reason metro density commands a valuation premium.

Adjacent segment | Diagnostics

Industry size and formalisation



FY24 and FY28E shown at the mid-point of CRISIL MI&A ranges (₹480-490 bn and ₹700-755 bn). Pathology is ~56% of the industry; organised chains hold only ~20-24% share, with standalone centres and hospital labs taking the rest — the formalisation runway is the whole investment case.

Metric	Dr Lal PathLabs FY26	Agilus (Fortis) FY26
Revenue	₹2,763 cr (+12.2%)	₹1,527 cr (+8.5%)
EBITDA	₹752 cr (+8.2%)	₹360 cr (+44.7%)
EBITDA margin	27.2%	23.6% (from 17.7%)
PAT	₹510 cr, margin 18.4%	Segment profit ₹242 cr
Volume	94.5 mn samples, 30.3 mn patients	40.8 mn tests
Network	312 labs, 7,727 collection centres	Pan-India lab network
ROCE	44% (excl. cash and investments)	n.d.
Working capital	Negative 26 days; ₹1,526 cr net cash	n.d.

Dr Lal guides FY27 revenue growth of 13-15% with EBITDA margins held at 27-28%, and has signalled that a price increase is possible but not imminent — a candid admission of how competitive pricing has become.

Recent consolidation and the strategic response

- Chains are moving up the value curve rather than fighting on price: Dr Lal launched Sovaaka, an AI-enabled premium wellness screening brand, and its Swasthfit preventive portfolio is already 27% of revenue.
- Geographic and inorganic infill continues — 14 new labs and over 1,100 collection centres added in FY26, plus the Shahbazkers acquisition in Mumbai and a new UAE subsidiary.
- Fortis has taken Agilus margins from 17.7% to 23.6% in a single year, largely on cost and network rationalisation rather than price — the clearest evidence that operating discipline, not pricing, is the available lever.

References: Company FY26 results (Dr Lal PathLabs, Fortis / Agilus); CRISIL MI&A via Max Healthcare investor presentation (May-26); Business Standard (Apr-26)